



Intraoral Scanning Instructions for Oral Appliance (Obstructive Sleep Apnea & Neuromuscular Orthotic)

Tooth Scanning Requirements

- Include 5 mm of gingiva beyond the CEJ throughout the entire mouth.
- In the upper anterior region (canine to canine), include at least 10 mm of buccal gingiva beyond the CEJ.
- Always strive to capture the full palate, whenever possible.

Bite Scans (Register Two Bites)

- Scan 1: Habitual bite (IC)
- Scan 2: Protrusive bite (forward vertical opening and mandibular advancement) using George Gauge, ProGauge, or similar device.

Guidelines for Bite Registration

- Assess the patient's habitual bite (IC).
- Observe if the patient's mandibular advancement is straight or has a side deviation.
- Compare the habitual bite (IC) with the working bite (Construction Bite).
- If there is a deviation, specify this in the physician questionnaire.



Important Considerations

- An unnatural bite position may cause muscle or joint pain, or long-term bite changes. Ensure the protrusive bite matches the patient's natural movement.
- Scan must capture the incisal edges of upper and lower teeth for proper alignment.
- Vertical opening in the anterior area should provide at least 4 mm opening in the posterior region, including the second molars.
- No need to increase opening if wisdom teeth limit the space. Note any interference (wisdom teeth, Curve of Spee, overeruption) in the physician form.

Tips for Protrusive Bite Scan

- Use the front part of the positioning device (no wider than lateral incisors).
- If the Vizbriz™ report is not available, scan in an edge-to-edge position with 5 mm anterior vertical opening.
- With older scanners, trim the distal fork of the George Gauge for better scan access.
- Record any real deviation (e.g., 2 mm left or 3 mm right) only if it's true, not from sliding. Document in the questionnaire.

For Day/Night Neuromuscular Orthotic (Airways Centric)

- Protrusive scan: edge-to-edge tooth position with 2 mm anterior vertical opening.

